Biotechnology

Pharvaris N.V. (PHVS)

EQUITY RESEARCH
September 14, 2026

Price: $38.02
Price Target: $65.00

Rating: Overweight

Key Statistics:

Symbol	NSQ: PHVS
52-Week Range	20.65 - 43.25
Market Cap	2,669.2
ADV (3 mo)	628,882
Shares Out (M)	70.2

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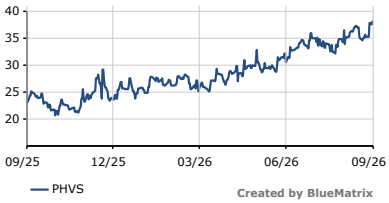
REV (£M)

FYE Dec	2025A	2026E	2027E
1Q	€0.0	€0.0A	€0.0
2Q	€0.0	€0.0A	€0.0
3Q	€0.0	€0.0E	€2.9
4Q	€0.0	€0.0E	€38.6
Year	€0.0	€0.0E	€41.5
P/S	-	-	64.3x

EPS

FYE Dec	2025A	2026E	2027E
1Q	€(0.85)	€(0.60)A	€(0.76)
2Q	€(0.83)	€(0.70)A	€(0.82)
3Q	€(0.60)	€(0.71)E	€(0.92)
4Q	€(0.72)	€(0.74)E	€(0.82)
Year	€(2.97)	€(2.76)E	€(3.33)

One-Year Price History



Company Update

Breakfast Bites @ Cantor HCC 2026

Following the very strong CHAPTER-3 readout, we hosted Pharvaris management at our conference, including a breakfast with investors where management 1) helped clear up any confusion regarding safety (liver enzyme elevations) and 2) provided color on financial strength and launch expectations.

While the strength of the Phase 3 data speaks for itself, in our view, we became incrementally more positive on the setup going forward.

Safety / liver enzyme elevations – important clarification following some confusion after the topline webcast. Management affirmed that the single Grade 4 event and the single SAE shown in the topline safety table were **the same event**, and importantly **not a liver event**. It occurred in an HAE patient with normal C1 inhibitor and a plasminogen mutation. This patient has a vocal-cord/laryngeal disorder with a history of laryngeal attacks and repeated intubations. The patient experienced a large (~90%) reduction in HAE attacks while on study and was able to go back to work as a pharmacist and rolled over into the open-label extension. This patient represents a success story for deucrictibant in Type 3 HAE.

Separately, there were **two Grade 3 liver-enzyme elevations**, both asymptomatic and without bilirubin elevation.

- One patient had a history of fatty liver disease and the other had prior androgen exposure. Since the study enrolled globally, including LATAM, APAC, and South Africa, where androgen use is more common and access to modern HAE prophylaxis is more limited, patients may have more fragile baseline liver function. One patient with a liver-enzyme elevation rolled into the open-label extension and remains under follow-up, with more detail expected in a future congress presentation/manuscript.
- There were additional Grade 1/2 liver-enzyme elevations, also asymptomatic and without bilirubin increases, but liver enzymes were **not among the three most common AEs**; those were URTI, nasopharyngitis and headache.
- It's important to note that LFT elevations have been observed in other HAE Phase 3 studies and typically constitute simply a note in the label in the laboratory abnormalities section.

Overall, management views liver safety as consistent with other HAE prophylaxis therapies.

Importantly, there were no GI or CV-related AE concerns.

Strength of financial position emphasized

Management is guiding current cash into mid-2028, which covers the initial portion of both launches, and the company has substantial non-dilutive alternatives.

Pharvaris has numerous options to finance the company further if warranted, including potential non-dilutive options, but is in no rush and can decide what to do from a position of strength.

Commercial guidance / launch readiness. Management has not yet decided exactly when or in what form to provide formal commercial guidance, although it expects investors to have substantially more visibility by the end of 2027. Importantly, the commercial organization is expected to be ~95% in place by YE26, with commercial product manufactured and the company positioned to launch IR promptly following the April 23, 2027 PDUFA, assuming approval.

Phase 3 follow-up: ACAAI in November, full manuscript potentially in 1Q27. PHVS plans to submit CHAPTER-3 data to the ACAAI meeting in the first week of November, which management expects will be the first venue for presentation of the fuller topline dataset to the medical community.

A manuscript shell is already prepared, and the team is populating the full dataset, allowing for internal review, submission and journal review/proof.

Management suggested publication could occur as early as 1Q27. The manuscript should contain the full efficacy and safety dataset, including timing of onset and resolution of LFTs.

Launch learning: Andembry provides a more relevant roadmap than Dawnzera. Management views Andembry's launch as the stronger recent HAE precedent, helped by CSL's ability to switch established Haegarda patients rather than build awareness and infrastructure from scratch.

PHVS believes deucrictibant may have a more advantageous setup since the IR deucrictibant sales force will already be in the market before the XR formally launches and the team will have time to establish prescriber relationships, capture patient leads and build familiarity.

By contrast, management characterized Ionis' Dawnzera launch as closer to starting from “ground zero.”

PHVS therefore effectively views the IR launch as the beginning of the XR launch rather than two separate commercialization efforts. This also feeds into pricing strategy as management will optimize the value of the overall deucrictibant franchise rather than IR economics alone.

Valuation

Our rating and price target are based on our DCF, which uses a 12% discount rate and assumes a 4% terminal growth rate beyond 2040E.

Risks

Clinical Risk: Preclinical safety finding that led to prior clinical hold turns out to be something more concerning than we think

Development Risk: Switch to XR formulation or other changes to patients' demographics in Phase 3 prophylaxis study lead to worse efficacy results and thus a less-competitive profile in Phase 3

Commercial Risk: If approved, Pharvaris is unable to capture market share in LTP HAE (too late in the game, undifferentiated vs. Orladeyo) or ODT HAE (late to market vs. Ekterly, ODT market mostly goes away due to LTP)

Company Description

Pharvaris N.V. is a clinical-stage biotechnology company developing oral small molecule therapeutics for hereditary angioedema (HAE). Lead asset, deucrictibant, recently completed Phase 3 studies: immediate release formulation for on-demand HAE treatment (positive data announced in December 2025) and extended release formulation for long-term prophylaxis of HAE.

Disclosures Appendix

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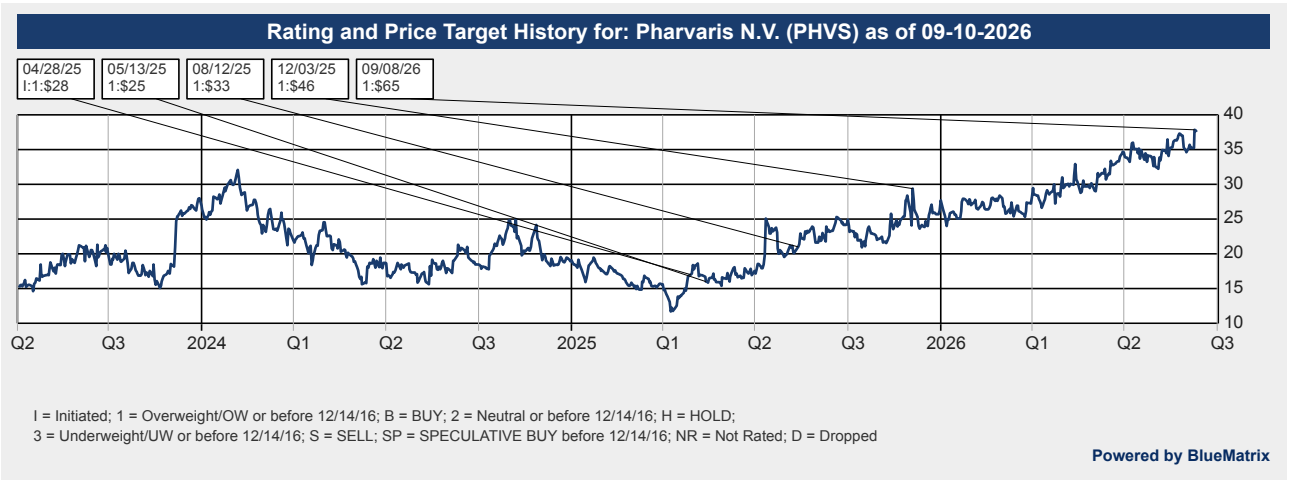
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Cantor

			IB Serv./Past 12 Mos.	
Rating	**Count**	**Percent**	**Count**	**Percent**
BUY [1/B]	**433**	**77.05**	**196**	**45.27**
HOLD [2]	**125**	**22.24**	**25**	**20.00**
SELL [SL/3]	**4**	**0.71**	**0**	**0.00**

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